

Clinic Intake Checklist

Reference packet for room setup and intake review

This packet is used by intake coordinators, nurses, assistants, and shift leads during a routine clinical room setup. The document is intentionally text-heavy so conversion tools must preserve real prose, not only isolated labels.

A faithful conversion keeps the title as one heading, preserves dot bullets as bullets, keeps numbered steps in order, maintains table columns, and leaves figures beside the captions that explain them.

Key setup checks:

- Confirm patient identity against the wristband and appointment record.
- Prepare the sterile tray before the clinician enters the room.
- Label the specimen bag with patient initials, visit date, and tray number.

Patient Verification

The coordinator asks the patient to state their full name and date of birth. Insurance, consent, allergy, and translator needs are checked before the clinical workflow begins. These checks are short, but they are important because the rest of the packet assumes that identity and consent steps are complete.

If the demographic record conflicts with the appointment record, intake pauses until the lead resolves the discrepancy. Staff must not overwrite the chart simply to clear the queue. The correction note records who reviewed the mismatch, which source was used, and the time the patient was allowed to proceed.

The verification section is often the first place where conversion systems lose ordinary prose because the text appears between checklists and tables.

Sterile Tray Setup

Figure 1 is intentionally local to the tray setup procedure.

The red square marker identifies the sterile tray zone. It belongs next to the setup steps because it tells the assistant where the sealed kit may be placed before the clinician opens it.

1. Wash hands before gloves and inspect the sterile field.
2. Put on sterile gloves before touching the tray contents.
3. Document the intake summary after the room is prepared.

Figure 1. The red square marks the tray zone used during Step 2.

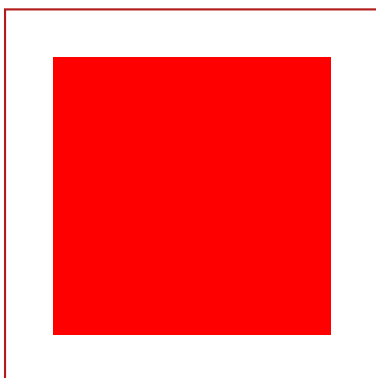


Figure 1. Red square tray-zone marker.

Box 1. Intake Reminders

Wet labels must be replaced before transport. Visitor restrictions are checked before the clinician begins. The specimen bag is read back to the clinician before it leaves the room.

These reminders are written as body text inside a visual box in the source packet. They should not become a stack of empty headings during conversion.

The box exists because these reminders are easy to skip during busy clinic hours, especially when rooms are reset quickly between appointments.

Specimen Handling

Specimen bags are staged on the side table until the clinician confirms that the sample has been collected. The assistant seals the bag, reads the label back to the clinician, and records the handoff time in the intake worksheet.

When the patient requires a translator, the translator name and session number are recorded next to the consent status. Missing translator evidence should be flagged during review because it can invalidate a downstream audit trail.

The source packet keeps these instructions as ordinary paragraphs so they can be searched and quoted in an audit report.

Daily Readiness Table

The readiness table links tasks, owners, and statuses. Converters should preserve its grid because the relationship between each task and owner matters.

Task	Owner	Status
Tray ready	Nurse	Done
Bag labeled	Assistant	Pending
Consent filed	Admin	Done
Translator booked	Coordinator	Done
Waste bin checked	Technician	Done

Figure 2. Mini status marker used in the readiness audit.



Safety Notes

The room should be reset after every encounter. Gloves and wipes are discarded immediately, the work surface is cleaned from the far edge toward the door, and the next tray is opened only after the surface is dry.

Wet surfaces can contaminate adhesive labels. Labels applied to damp bags may detach during transport and break chain-of-custody evidence. This paragraph is important because it explains why the label replacement step is not administrative trivia.

The safety section should remain near the handling section, not drift into an appendix.

Shift Handoff

At the end of the shift, the lead compares the intake worksheet against the sample transport manifest. Exceptions are summarized in the handoff note and sent to the next shift lead before the room is closed.

Escalation timing matters. A wet-label replacement must be documented before the transport manifest is sealed. If chain-of-custody evidence is corrected after sealing, the correction must identify the original recorder, the reviewer, and the reason for the late change. This large source-only paragraph is deliberately absent from the broken candidate.

The handoff note is short, but it is the document that makes the next shift safe.

Audit Review Guidance

Reviewers compare converted Markdown against the source packet by checking semantic fidelity rather than cosmetic perfection. A conversion can be acceptable with minor whitespace differences, but it is not acceptable if steps are reordered, captions drift away from figures, tables flatten into ambiguous text, or source-only paragraphs disappear.

The judge should prefer evidence. It should name the problem, explain why it affects meaning, and avoid vague statements such as format looks bad.

This page provides more realistic surrounding prose so the benchmark does not reward models that only search for obvious marker tokens.

Appendix: Retention and Contact Notes

Completed intake worksheets are retained with the daily manifest. The room log is retained separately because it contains supply exceptions that are not patient-specific. When a reviewer needs clarification, the shift lead is contacted first, followed by the clinic manager if the lead is unavailable.

The appendix is ordinary operational text. It should survive conversion as readable prose with stable heading hierarchy and without repeated running headers.